

Mattau Health

Health worker

Health information for every village in Papua New Guinea.

More topics

- Safe Motherhood
- TB
- Malaria
- Healthy Family, Healthy Community
- Healthy Village
- Policies
- Child Health
- Family Planning
- Clean Water and Hygiene
- How to Teach
- AIDS

This site gives general health information. If you or someone in your family is sick, go to your nearest health centre or talk to your village health worker.

Safe Motherhood

You are the first link to care for pregnant women and new mothers. Identify, counsel, refer and follow up. Your home visits do not replace antenatal visits at the health facility.

For health workers

Key messages

- Identify pregnant women in your area. Signs are a missed period, bigger breasts and darker nipples, feeling sick, passing urine often, and a growing belly. Create awareness and refer her to the facility to confirm, and keep it confidential.
- Encourage at least four ANC visits. Your monthly home visits do not count as ANC. Only a midwife, nurse, doctor or CHW at the facility or an outreach clinic can do ANC.
- Ask what stops her from going: no money for transport, distance, a bad past experience, nowhere to stay, or a family

that does not support her. Work with the family and the ward health committee on solutions.

- At home visits, look at her Helt Buk Bilong Ol Meri: due date (40 weeks after her last period), weight, blood pressure, iron and folic acid tablets, and tests done.
- Support the ANC care package: iron and folic acid, two tetanus doses, malaria prevention medicine (Fansidar, three doses at least a month apart from the second trimester), a bed net, and testing for HIV, syphilis and TB.
- Help every family make a birth plan: birth at the facility, transport and its cost, who goes with her, what to take (blanket, towel, clothes for mother and baby), and when to move near the facility.
- Check for danger signs at every monthly visit and refer urgently.
- After the birth, encourage postnatal checks on day 1, 3, 7 and 14 and at 6 weeks. Support breastfeeding within the first 90 minutes and only breast milk for six months.

Refer urgently if she has

- Any vaginal bleeding in pregnancy, or heavy bleeding after birth
- Severe headache, blurred vision, fits, or sudden swelling of face, fingers, legs or feet
- Fever or a very hot body
- Baby moving much less or not at all
- Severe belly pain, or vomiting in late pregnancy
- Waters leaking, or labour pains before the due date
- After birth: smelly vaginal discharge, lasting belly pain, breast pain or swelling
- After birth: fits, or she is withdrawn and not interested in her baby

Work with the community

- Help the ward health committee arrange transport for pregnant women, and an emergency fund.
- Talk with husbands and elders so families support ANC and birth at the facility.

- Keep all information about women private.

Run a village session

- 1.** Meet in a shaded, central place at a time when families can come. Greet everyone and say what today's topic is.
- 2.** Ask what people already know and what they have heard. Listen first.
- 3.** Share three key messages from this page in your local language. Use pictures and real examples from the village.
- 4.** Ask people to say the messages back in their own words, and answer questions.
- 5.** Agree on one action the village will take this month. Refer anyone who is unwell, and record who came.

General information only. Follow the advice of your health centre and current national guidelines. Based on the National Department of Health Village Health Assistant (VHA) training curriculum.

TB

You cannot diagnose TB. Find people with signs, send them for a test, and support them through treatment. Community support is a big part of the cure.

For health workers

Key messages

- Ask about cough at every home visit. Refer anyone with a cough for two weeks or more, or a child with a cough for 14 days or more, for proper diagnosis and treatment.
- At home visits, encourage patients on long-term treatment to keep taking their medicine, and help them find a treatment supporter.
- Watch for side effects and missed doses, and tell the health facility early.
- Check the people who live with a TB patient, especially children.

- Pregnant women are screened for TB at antenatal care. Encourage them to attend.
- Fight stigma. TB is curable, and patients need support from family, not fear.

Refer straight away if a patient has

- Coughing blood
- Difficulty breathing
- Missed medicine for several days
- Serious side effects such as yellow eyes or repeated vomiting
- A child in the household with a long cough or weight loss

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Malaria

You help families prevent malaria and get to a test quickly. Malaria medicine is given only to people with a confirmed test, by a trained health worker.

For health workers

Key messages

- Send anyone with a fever for a test within 24 hours, using a rapid test or blood slide at the health facility, or through a trained community malaria volunteer.
- Do not give malaria medicine for fever alone. Refer pregnant women with signs of malaria to the facility for testing and treatment.
- Explain why the full three days of treatment must be finished, and that the medicine is free.
- Help make sure every household gets a net when nets are given out (about every three years), especially pregnant women.

Help families hang nets properly.

- At home visits, check that nets are hung and have no tears or holes.
- Teach families to clear bush and empty puddles. Tell pregnant women about malaria prevention tablets at ANC.
- Record your referrals and work with the ward health committee and community malaria volunteers.

Refer immediately if the patient has

- Fits, confusion or unusual sleepiness
- Cannot drink or keeps vomiting
- Very pale skin or yellow eyes
- Fast or difficult breathing
- Fever in a pregnant woman or a baby

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Healthy Family, Healthy Community

The Helti Famili checklist shows 16 things a healthy family has. Use it with each family on home visits, and help them choose what to improve first.

For health workers

Key messages

- Sit with the family and go through the 16 items below. Mark each one: Red if the family has none, Yellow if they have some but not enough, Green if they have enough.
- Look at the yellow and red items together. Ask what makes each one hard, and what small step the family can take first.
- Ask the person to say their commitment out loud, ideally in front of family. Check progress on your next visit.

- Visit families with new mothers and babies, sick people, and children not growing well first.
- Visits last about 30 minutes. Invite husbands, parents and older children to join. Do the conversation first, then care.
- Bring the household monitoring form and the Mother and Child Health Record Books.
- Keep everything you learn in the household private.

Helti Famili checklist

Tick one box for each: none, some or enough.

	None	Some	Enough
1. Clean house			
2. Clean water			
3. Toilet			
4. Rubbish pit			
5. Foods that help the body grow			

	None	Some	Enough
6. Many kinds of fruit and vegetables			
7. Foods that give strength			
8. Small garden near the house			
9. Children and women are vaccinated			
10. Beds to sleep on			
11. Plates, spoons, cups and saucepans			
12. Lamp			
13. Mosquito net, clothes, blankets and pillows			
14. Spade, axe and bush knife			
15. A way to earn money			
16. Peace and good living together			

Tell the health facility if you see

- A child who is very thin, has swollen feet, or is not growing
- Many people in one village with the same sickness
- A household with no safe drinking water
- A pregnant woman who has not been to antenatal care

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Healthy Village

Community action and participation is how a village takes ownership of its health. Trust and ownership are what make health work last.

For health workers

Key messages

- Use the six steps of community action: 1. Get to know the community. 2. Work out community needs. 3. Decide what to do first. 4. Plan how to do it. 5. Take action together. 6. How did we do?
- Work with the ward health committee, village leaders, churches and women's groups.
- Use the Helti Ples checklist with village leaders to see what the village has and what it needs.
- Draw a community map and update it at least every 6 months. Use it to plan where to focus.

- Plan your work for at least three months, including a mix of health promotion activities.
- Link the village to the health centre and to outreach clinics. Collect simple health data and report it every month.
- The Healthy Islands idea means making the places where people live, work, learn and play healthy places.

Helti Ples checklist

Tick one box for each: none, some or enough.

	None	Some	Enough
1. Village health committee			
2. Clean houses and good air			
3. Clean water			
4. Clean village everywhere			
5. Good health services			
6. A place of worship			
7. A good school			
8. Working together			

	None	Some	Enough
9. Small gardens near houses			
10. Good roads and transport			
11. Many ways to earn money			
12. Peace and good living together			
13. Community bank			
14. MCH clinic			
15. Change bad traditional customs			
16. Teach people about health			

Raise with the health facility if

- There is an outbreak or many people are sick at once
- The village cannot reach the health centre because of the road or river
- Supplies you rely on have run out

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Policies

The policies behind your work, in plain language.

For health workers

Key messages

- Papua New Guinea's Vision 2050 aims for a smart, wise, fair, healthy and happy society. The National Health Plan 2021 to 2030 turns this into action.
- Key Result Area 1 of the plan is about developing VHAs to strengthen village and community health services. Village Health Assistance is Intervention No. 1.
- The VHA Policy sets up village health workers as a formal part of the health system. VHAs are paid according to government standards and skill level.
- VHAs are the first point of contact for health promotion, awareness and disease prevention. They link rural and remote communities to the health system.

- VHAs do not give curative treatment. They promote health, give first aid information, and refer.
- Always keep client information private and treat people with respect.

Who is a VHA

- A woman or man chosen by the community, with the ward health committee helping the process, trained and supervised.
- At least 25 years old, of sound mind, and able to walk distances and carry supplies. Grade 10 to 12 is preferred, and at least one VHA in the team must be able to read.
- Has lived in the ward for three years, is respected and responsible, and works well with health workers.
- A minimum of 2 VHAs per ward.

Working hours

- 10 working days each month, out of 22, with 8 hours a day.

- You can choose your own schedule as long as the basic parts of the work are done.

Who supervises you

- Health facility in-charge: technical support, at least monthly at first and every 3 months after.
- Ward councillor and ward health committee: administrative support, about every two weeks.
- Provincial health authority and district health managers: quarterly meetings, approve work plans and pay.
- In some districts, church health organisations also support VHAs.

In an emergency

- Advise the person to go to the nearest health facility at any level. Where you can, go with them.
- Call 111, or call your technical supervisor.
- Emergencies include: not able to wake, seizure or severe head injury, chest pain,

stroke or face or arm weakness, severe trouble breathing, severe bleeding, severe burns, major injury, severe allergic reaction, or a serious assault.

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Child Health

You monitor the growth of every child under five, check vaccinations, and refer sick and malnourished children. You do not give curative treatment.

For health workers

Key messages

- At home visits, check the Child Health Record Book. If a vaccine is missed, ask why, explain that vaccines are safe, effective and free, and offer to go with the family.
- Vitamin A is given every six months from 6 months, and deworming from 12 months. Encourage multiple micronutrient powder for children 6 months to 5 years where available.
- Growth monitoring: use the MUAC tape on children. Below 11.5 cm (red) is severe wasting. Yellow also needs referral. Swelling of both feet is a danger sign. Refer all of these.

- Counsel on feeding: only breast milk to 6 months. From 6 to 23 months children should eat at least five of the eight food groups each day, and from 12 months about five times a day.
- Mild cough, diarrhoea and fever can be managed at home. Continue breastfeeding, give extra fluids, and keep the child fed. Refer when there is a danger sign.
- Mild vaccine side effects, such as a sore arm or fever, go away in 1 to 2 days. Refer if worse after 2 days. Trouble breathing, swelling of face or throat, bad rash all over, or fits need the health centre immediately.
- Support positive parenting: eye contact, smiles, cuddles and praise. Help caregivers notice hunger, illness and distress. Encourage parenting without violence.
- Work with the ward health committee to find solutions to barriers such as distance, cost and fear of hospitals.

Refer at once if a child has

- Fast breathing (50 or more breaths a minute at 2 to 12 months), chest indrawing, or cough for 14 days or more
- Diarrhoea for 14 days or more, blood in the stool, or signs of dehydration
- Fever for 7 days or more, or with fits, or a stiff, unconscious or hard to wake child
- Not able to drink or breastfeed, or vomits everything
- Red or yellow MUAC, or swelling of both feet
- Newborn: not feeding, floppy, too hot or too cold, yellow skin in the first day, pus in the eyes for 2 days, or fits

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Family Planning

You give information, counsel, and refer. Help women and men make their own informed choice, and listen to what stops them.

For health workers

Key messages

- Explain family planning as a voluntary, informed choice about when to have children, how many, and the time between them.
- Know the methods and where they are available: condom, implant, injection, IUD, pills, ovulation method, tubal ligation, vasectomy, and the emergency pill.
- Say clearly that condoms are the only method that protects against both pregnancy and STIs including HIV.
- Listen for barriers: fear of side effects, a partner who does not agree, myths, distance. Use the steps of behaviour

change communication to find solutions.

- Take special care to be respectful and private with young people, women who had a near-miss or emergency in a past birth, and women with disabilities or illness.
- Refer to the health facility for the method chosen.
- All family planning is free at government health facilities. Report any health worker selling contraceptives.

Refer if

- A woman has a problem or side effect with a method
- Unprotected sex and she wants the emergency pill within 5 days
- Signs of a sexually transmitted infection

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Clean Water and Hygiene

Poor hygiene and unsafe water make people sick. Talk with families and lead village action to make water and sanitation safe.

For health workers

Key messages

- Teach handwashing at five key times: after the toilet or cleaning a child, before cooking, before feeding children, before eating, and before breastfeeding.
- Teach that clear water is not always safe. Boiling is a local way to treat water.
- Help families find local options for safe water, and for safe toilets and rubbish pits.
- Show how to build a tippy tap for handwashing with running water.

- In dialogue with families, ask how germs get from pekpek to the mouth, and what they can do to stop it.
- Mobilise the village to act on water, toilets and rubbish, using the community action steps.

Tell the health facility if

- Many people have diarrhoea at the same time
- A child has bloody diarrhoea or diarrhoea for 14 days or more
- The village has no safe water

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How to Teach

Health promotion means giving people the skills and knowledge to take control of their own health. It works best as a conversation, not a lecture.

For health workers

Key messages

- Behaviour change is a process. It does not happen overnight.
- Use dialogue, not lectures. Ask questions, listen, and let people decide. Link the healthy behaviour to clear benefits for their lives.
- Listen actively. Sit at the same level, lean in, nod, do not interrupt, and repeat back what you heard. Ask open questions such as what makes this difficult for you.
- Help the person prepare and find supporters, choose one small step they can do, and say their commitment out loud, ideally in front of family and friends.

- Home visits: plan about 30 minutes, invite the whole household, do the conversation first and care afterwards, and keep it private. Visit new mothers, sick people and children not growing well first.
- Group dialogue: sit in a circle, agree ground rules, give everyone a turn, thank people who talk a lot and invite quieter people, and end with each person's commitment.
- Use pictures, posters and job aids from the NDoH. Speak slowly, clearly and in the local language.
- Follow up. Ask about their commitment on your next visit and celebrate progress.

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AIDS

You educate, support people to get tested and stay on treatment, and fight stigma. Above all, keep everyone's information private.

For health workers

Key messages

- All pregnant women should be tested for HIV and syphilis at ANC. Explain the steps of ePTCT: take medicine regularly, give birth at the facility, the baby gets medicine within 72 hours, only breast milk for six months, the baby is tested at 6 weeks and 18 months.
- Never share confidential information, even with the person's spouse, family or your own family. Share only with their health worker or your supervisor when needed.
- Fight stigma: educate the community, correct myths, encourage support, and be a role model. Stigma stops people getting

tested and treated.

- Support clients: help them find a treatment supporter, watch for side effects, encourage them to keep taking ART, and remind them they can live a long normal life.
- Trace clients who stop coming for medicine, confidentially and only if they have agreed to your support.
- Encourage good food, and discourage alcohol and smoking. Encourage partners to be tested.
- Link clients to the health facility when needed. Watch for signs of depression and seek help.

Refer if a person has

- Not started or stopped ART
- Long fever, diarrhoea or weight loss
- A cough of two weeks or more (possible TB)
- Signs of depression or thoughts of self-harm

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